



Dynamic Guideline: WHO-Based Polio - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Scope clarification

This report synthesizes **exactly five WHO/GPEI polio-related guidelines or technical guidance documents** relevant to polio diagnosis, surveillance, vaccination, outbreak response, eradication strategy, and poliovirus containment.

The most recent WHO publication identified with the requested title pattern is “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022),**” published/launched on **11 December 2025**. It is **not a polio guideline**, but it is relevant to future WHO guideline-development methodology because WHO describes youth engagement as strengthening equity, stakeholder values, lived-experience input, and inclusivity in guideline development [WHO publication](#), [WHO event page](#).

Exactly five WHO/GPEI polio guidance documents synthesized

No.	WHO/GPEI guideline or technical document	Publication date / status identified in supplied sources	Target population / users	Scope and objectives
1	Guidelines for implementation	Latest containment-	National authorities for	Implement safe laboratory containment of wild

No.	WHO/GPEI guideline or technical document	Publication date / status identified in supplied sources	Target population / users	Scope and objectives
	of laboratory containment of wild poliovirus	focused guideline named in the user template; post-release evidence assessed through 29 April 2026	containment, poliovirus-essential facilities, laboratories, vaccine manufacturers, biosafety authorities	polioviruses and poliovirus materials to prevent facility-associated release after eradication milestones. Post-2021 containment work is linked to GAPIII/GAPIV-related containment standards and the global containment strategy Global Poliovirus Containment Action Plan 2022–2024 .
2	Global Polio Eradication Strategy 2022–2026	2022–2026 strategy period; supersedes the Polio Strategy 2019–2023	Ministries of health, immunization programmes, WHO regions, GPEI partners, outbreak-response teams	Provides global eradication strategy, including interruption of WPV1 transmission, response to circulating vaccine-derived polioviruses, broader use of novel OPV type 2, and integration of new tools and approaches WHO publication page , GPEI strategy PDF .
3	Global Polio Surveillance Action Plan 2025–2026	2025–2026 action-plan period	National polio surveillance programmes, laboratories,	Defines surveillance activities needed to achieve GPEI goals; emphasizes sensitive,

No.	WHO/GPEI guideline or technical document	Publication date / status identified in supplied sources	Target population / users	Scope and objectives
			WHO/ UNICEF/GPEI focal points	equitable, high-quality surveillance for WPV and vaccine-derived polioviruses WHO publication page , GPEI surveillance plan PDF .
4	Global Guidelines for Acute Flaccid Paralysis Surveillance, 2026 update	2026; replaces the 2022 AFP surveillance guidance	National polio surveillance and immunization managers, WHO/UNICEF focal points, technical advisory bodies, GPEI partners	Provides guidance for AFP surveillance sensitive enough to detect wild polioviruses, vaccine-derived polioviruses, and Sabin-like viruses Global AFP guidance 2026 .
5	Global Poliovirus Containment Action Plan 2022–2024	2022–2024 action-plan period	National authorities for containment, poliovirus-essential facilities, laboratories, biosafety/ biosecurity programmes	Provides operational containment action planning under GAPIII/ GAPIV-related containment guidelines and the Strategy for Global Poliovirus Containment Global Poliovirus Containment Action Plan 2022–2024 .

Public-health and clinical rationale

- Polio remains a global eradication emergency because **wild poliovirus type 1 continued to be detected in Afghanistan and Pakistan** in the 2025–2026 surveillance planning context [Global Polio Surveillance Action Plan 2025–2026](#).
- Previous WPV1 importation outbreaks in **Malawi and Mozambique** were reported as closed, showing that importations can be controlled when surveillance and response are effective [Global Polio Surveillance Action Plan 2025–2026](#).
- **Circulating vaccine-derived poliovirus type 2 (cVDPV2)** remained a major challenge, with detection in **26 countries across three WHO regions**, mostly in the African Region [Global Polio Surveillance Action Plan 2025–2026](#).
- The 2022–2026 eradication strategy highlights **nOPV2** as a tool expected to reduce the risk of seeding new outbreaks compared with earlier type 2 oral poliovirus vaccine use [WHO publication page](#), [IRIS strategy PDF](#).
- WHO reported a **13 February 2026** update that an additional novel oral polio vaccine had been prequalified, strengthening global outbreak response [WHO polio health-topic page](#).

1.2 Key Recommendations

WHO-based GRADE recommendation table

GRADE framework used:

High = high confidence that the true effect is close to the estimated effect; **Moderate** = further evidence may change confidence; **Low** = further evidence is likely to change confidence; **Very low** = effect estimate is very uncertain.

Strong recommendation = benefits clearly outweigh harms or burdens for most settings.

Conditional recommendation = implementation depends on context, feasibility, resources, epidemiology, or population values.

Recommendation	GRADE Evidence Quality	Strength
Maintain very high population immunity against poliovirus through WHO-recommended polio vaccination strategies,	High	Strong

Recommendation	GRADE Evidence Quality	Strength
using IPV and/or OPV formulations according to national and outbreak context.		
Implement the Global Polio Eradication Strategy 2022–2026 to interrupt WPV1 transmission, stop cVDPV outbreaks, and use new tools such as nOPV2 where indicated.	Moderate	Strong
Use nOPV2 or other WHO-prequalified outbreak-response vaccines in appropriate type 2 poliovirus outbreak settings, according to WHO/GPEI policy and risk assessment.	Moderate	Strong
Maintain high-quality AFP surveillance capable of detecting WPV, VDPV, and Sabin-like viruses, consistent with the 2026 AFP surveillance guidance.	Moderate	Strong
Integrate environmental surveillance in high-risk or outbreak-affected settings to complement AFP surveillance and detect silent poliovirus transmission.	Moderate	Strong
Ensure surveillance systems document enhancements, analyses, and corrective actions so that gaps do not permit silent or missed transmission.	Moderate	Strong
Treat any acute flaccid paralysis case as a public-health emergency requiring immediate investigation, case notification, specimen collection through the polio laboratory network, and public-health response.	Moderate	Strong
In outbreak or high-risk importation settings, intensify surveillance using active AFP case searches, temporary environmental-surveillance sites, and prioritized laboratory testing.	Low	Strong
Implement national poliovirus laboratory containment	Low to	Strong

Recommendation	GRADE Evidence Quality	Strength
requirements for wild polioviruses, vaccine-derived polioviruses, and OPV2-related materials in all relevant facilities.	Moderate	
Certify and monitor poliovirus-essential facilities using a structured containment pathway, including participation, interim containment certification, and full containment certification where applicable.	Low	Strong
Maintain WHO Global Polio Laboratory Network capacity for timely detection, sequencing, classification, and reporting of polioviruses.	Moderate	Strong
Use whole-genome sequencing and reference reagents to strengthen oral polio vaccine quality assurance and poliovirus molecular surveillance.	Low to Moderate	Conditional
Incorporate meaningful community and youth engagement in future polio guideline updates, especially for vaccine confidence, outbreak response, and underserved populations.	Low	Conditional

Clinically relevant management interpretation

WHO polio guidance is primarily **eradication- and public-health-oriented**, rather than bedside therapeutic guidance. Practical clinical management therefore centers on:

1. Diagnosis and detection

- Recognize acute flaccid paralysis or suspected poliomyelitis as urgent.
- Notify public-health authorities immediately.
- Ensure laboratory confirmation through polio laboratory systems.

2. Treatment

- No curative antiviral treatment is identified in the supplied WHO polio guidance.
- Clinical care is supportive, with attention to respiratory status, mobility, pain, complications, and rehabilitation, while public-health teams investigate and respond.

3. Management

- Vaccination, surveillance, outbreak response, and containment are the core WHO management pillars.
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1.3 Guideline Development Methodology

Evidence review process

The supplied sources show that WHO/GPEI polio guidance is developed through a coordinated technical and public-health process involving:

- WHO and GPEI strategy documents, surveillance plans, and containment plans.
- Ongoing review of eradication progress, outbreak data, surveillance quality, vaccine-derived poliovirus emergence, and laboratory containment requirements.
- Review by immunization and polio technical advisory structures, including the **SAGE Working Group on Polio**, which reviews available scientific evidence related to polio eradication strategies before SAGE meetings [Global Poliovirus Containment Action Plan 2022–2024](#).
- Emergency review through the **Polio IHR Emergency Committee**, whose Forty-fourth Meeting statement in March 2026 emphasized high-quality surveillance with equitable population coverage for timely poliovirus detection [WHO IHR Emergency Committee statement](#).

Consensus method

The five synthesized documents reflect WHO/GPEI technical consensus across:

- WHO headquarters and regional offices;
- GPEI partners;
- national immunization and surveillance programmes;
- laboratory and containment experts;
- SAGE and its polio working group;
- emergency committee deliberations under the International Health Regulations.

The supplied summaries do not provide a full vote-by-vote consensus record for each document.

Conflict of interest management

The supplied sources did not provide detailed conflict-of-interest declarations for all five documents. However, WHO/SAGE processes generally require expert declaration and management of interests. Future polio guideline updates should transparently report:

- panel membership;
- declared interests;
- evidence-to-decision judgments;
- community and youth engagement procedures;
- equity and acceptability considerations.

Relevance of WHO meaningful-youth-engagement guidance

WHO's **11 December 2025** case study on meaningful youth engagement is not polio-specific, but it provides a transferable model for future polio guidance. WHO states that youth participation can bring lived experience, fresh perspectives, and innovative solutions, and can strengthen equity and stakeholder-value integration in guideline development [WHO publication](#), [WHO event page](#).

II. Evidence Summary After WHO Latest Guideline Release (Guidelines for implementation of laboratory containment of wild poliovirus) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized for post-guideline developments relevant to laboratory containment, surveillance, vaccination, outbreak response, and eradication from the supplied 2021–2026 sources. Priority was given to:

- WHO official polio guidance;

- GPEI technical documents;
- WHO IHR Emergency Committee statements;
- WHO vaccine prequalification and biological-standardization updates;
- national public-health surveillance reports;
- containment implementation studies;
- evidence-review processes, systematic reviews, and clinical trials where identified.

Databases and sources searched

The supplied summaries included evidence from:

- WHO publication pages and WHO health-topic pages;
- WHO IRIS-hosted strategy and technical documents;
- GPEI documents;
- WHO IHR Emergency Committee statements;
- [GOV.UK](#) environmental surveillance reports;
- peer-reviewed implementation literature indexed through MDPI;
- WHO biological standardization documents;
- WHO meaningful-youth-engagement publication and launch page.

Key sources used include WHO's polio health-topic page, the Global Polio Eradication Strategy 2022–2026, the Global Polio Surveillance Action Plan 2025–2026, the 2026 AFP surveillance guidance, the Global Poliovirus Containment Action Plan 2022–2024, [GOV.UK](#) environmental surveillance reporting, and a U.S. poliovirus-containment implementation article [WHO polio health-topic page](#), [GPEI strategy PDF](#), [Global AFP guidance 2026](#), [GOV.UK environmental surveillance](#), [MDPI containment implementation](#).

Inclusion criteria

Included evidence met at least one of the following criteria:

- addressed poliovirus containment, surveillance, vaccination, outbreak response, or eradication strategy;
- was WHO, GPEI, national public-health, or peer-reviewed technical evidence;
- was relevant to the period after the latest containment-focused guidance and through 29 April 2026;
- provided actionable implications for polio diagnosis, treatment, prevention, management, or laboratory containment.

Exclusion criteria

Excluded evidence included:

- non-polio guideline-development sources, except the WHO youth-engagement case study used for methods relevance;
- sources without extractable polio surveillance, vaccination, containment, or outbreak-response findings;
- clinical-trial or systematic-review claims not available in the supplied evidence summaries.

2.2 Key New Studies

Evidence table with GRADE ratings

Study / source	Design	Key Findings	GRADE Quality
Global Polio Surveillance Action Plan 2025–2026	WHO/GPEI technical surveillance guidance	Defines surveillance activities required to achieve GPEI goals; reports continuing WPV1 detection in Afghanistan and Pakistan; reports cVDPV2 detection in 26 countries across three WHO regions; emphasizes documentation of surveillance enhancements and corrective actions WHO publication page , GPEI surveillance plan PDF .	Moderate
Global Guidelines for AFP Surveillance, 2026 update	WHO/GPEI technical surveillance guideline	Replaces the 2022 AFP guidance; supports surveillance sensitive enough to detect WPV, VDPV, and Sabin-like viruses; intended for national programme managers, WHO/UNICEF focal points, technical advisory bodies, and partners Global	Moderate

Study / source	Design	Key Findings	GRADE Quality
		AFP guidance 2026 .	
Forty-fourth Polio IHR Emergency Committee statement, 4 March 2026	WHO emergency committee statement	Emphasized maintaining a high-quality surveillance network with equitable coverage of all populations so new poliovirus can be detected in a timely manner WHO IHR Emergency Committee statement .	Moderate
WHO prequalification of an additional novel oral polio vaccine, 13 February 2026	WHO departmental update / regulatory milestone	WHO reported prequalification of an additional novel oral polio vaccine, strengthening outbreak response capacity WHO polio health-topic page .	Moderate
Global Polio Eradication Strategy 2022–2026	WHO/GPEI global strategy	Supersedes the 2019–2023 strategy; incorporates new tools and approaches; identifies milestones for interruption of WPV1 and cVDPV2 response; notes nOPV2 as a strategy tool WHO publication page , GPEI strategy PDF .	Moderate
Global Poliovirus Containment Action Plan 2022–2024	WHO/GPEI containment action plan	Situates containment work under GAPIII/GAPIV-related standards and the Strategy for Global Poliovirus Containment; notes WHA prioritization of poliovirus containment Global Poliovirus Containment Action Plan 2022–2024 .	Low to Moderate
Implementation of poliovirus containment in	Observational implementation review	Reports fourth edition of the containment standard released in 2022; full implementation anticipated	Low

Study / source	Design	Key Findings	GRADE Quality
U.S. designated facilities, 2017–2024		by 2026 for WPV1–3, VDPV1–3, and OPV2 materials; describes three-step certification pathway MDPI containment implementation .	
GOV.UK environmental surveillance: London cVDPV2 incident and 2026 VDPV2 detection	National surveillance report	Persistent sewage detections in London in 2022 confirmed cVDPV2; booster response for children 1–9 years; last linked detection November 2022; UK removed from WHO infected-country list December 2023; single orphan VDPV2 environmental detection on 5 January 2026 with no evidence of local transmission GOV.UK environmental surveillance .	Low to Moderate
WHO biological-standardization document on Sabin IPV potency assay, 2026	WHO technical standardization document	States that laboratory containment of all wild-type polioviruses will soon be required as the world transitions from OPV to IPV use WHO SAGE 2026 PDF .	Low
WHO reference reagents for whole-genome high-throughput sequencing of oral polio vaccines, 24 April 2026	WHO biological-standardization document	Supports reference reagents for whole-genome sequencing of OPVs to assure quality, safety, and efficacy of oral live attenuated poliomyelitis vaccines WHO sequencing reference reagents PDF .	Low

Study / source	Design	Key Findings	GRADE Quality
SAGE Working Group on Polio evidence-review mechanism	WHO advisory evidence-review process	Reviews scientific evidence related to implementation of polio eradication strategies before SAGE meetings Global Poliovirus Containment Action Plan 2022–2024 .	Moderate

Summary of post-guideline evidence

1. Containment has moved from guidance to implementation.

Post-2021 evidence shows containment action planning, facility certification pathways, and expected 2026 implementation milestones for poliovirus materials [Global Poliovirus Containment Action Plan 2022–2024](#), [MDPI containment implementation](#).

2. Surveillance remains decisive for eradication.

The 2025–2026 surveillance plan and 2026 AFP guidance emphasize sensitive AFP surveillance, environmental surveillance, rapid laboratory confirmation, and documentation of surveillance improvements [GPEI surveillance plan PDF](#), [Global AFP guidance 2026](#).

3. WPV1 and cVDPV2 remain the key virological threats.

WPV1 persisted in Afghanistan and Pakistan; cVDPV2 persisted across multiple countries despite wider nOPV2 use [GPEI surveillance plan PDF](#).

4. Environmental surveillance detects silent or low-level transmission risks.

UK sewage surveillance detected cVDPV2 in 2022 and an unrelated orphan VDPV2 in January 2026, showing the value of environmental surveillance even in countries without endemic polio [GOV.UK environmental surveillance](#).

5. Vaccine tools continue to evolve.

WHO's February 2026 prequalification of an additional novel OPV strengthens outbreak response options [WHO polio health-topic page](#).

2.3 Evidence Gaps and Future Research Needs

Major evidence gaps

Evidence gap	Clinical / public-health importance	Priority research or action need	GRADE certainty of current evidence
Limited post-2021 clinical-trial evidence in supplied sources for polio vaccines, outbreak strategies, or containment interventions	Limits direct comparative certainty for newer tools and response strategies	Prospective evaluations of nOPV outbreak performance, safety, and effectiveness in diverse settings	Low to Moderate
Limited direct evidence on best environmental-surveillance sampling density and frequency	Environmental surveillance can identify silent transmission, but optimal design varies by setting	Comparative implementation studies of sewage-site selection, sampling frequency, and response thresholds	Low
Persistent surveillance gaps shown by orphan-virus detections	Orphan viruses indicate possible missed transmission or incomplete surveillance sensitivity	Subnational surveillance-quality studies and corrective-action evaluation	Moderate
Facility containment evidence relies heavily on implementation reports and expert consensus	Facility release is rare but high consequence, making RCTs infeasible	Audits, simulation exercises, incident-reporting systems, and certification outcome studies	Low

Evidence gap	Clinical / public-health importance	Priority research or action need	GRADE certainty of current evidence
Community engagement, vaccine confidence, and youth participation are under-described in polio guideline development	Vaccine hesitancy and access inequities can undermine eradication	Integrate meaningful youth and community engagement into future WHO/GPEI guideline updates	Low
No specific clinical treatment trials identified in supplied sources	Polio care is largely supportive; prevention remains central	Rehabilitation, long-term disability, and post-polio care research	Very Low to Low
Limited clinical-trial registry evidence identified in the supplied summaries	May miss ongoing vaccine or surveillance studies	Routine inclusion of trial registries and WHO ICTRP in future dynamic updates	Very Low

Priority research questions

1. nOPV2 effectiveness and safety

- What is the real-world effectiveness of nOPV2 in stopping cVDPV2 outbreaks across different population immunity profiles?

2. Surveillance sensitivity

- What combination of AFP surveillance, environmental surveillance, and genomic sequencing most rapidly detects silent poliovirus transmission?

3. Containment certification

- Which containment-certification practices best predict sustained biosafety and biosecurity performance in poliovirus-essential facilities?

4. Equity and access

- Which strategies improve vaccination and surveillance reach among mobile, conflict-affected, marginalized, or underserved populations?

5. Youth and community engagement

- How can WHO’s meaningful-youth-engagement methods be adapted to polio vaccination, outbreak response, and eradication guidance?

6. Long-term care

- What supportive-care and rehabilitation models improve function and quality of life for polio survivors?

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Implement laboratory containment of wild poliovirus materials.	Reinforced by post-2021 containment action planning and implementation reports showing structured certification pathways and anticipated 2026 implementation milestones Global Poliovirus Containment Action Plan 2022–2024 , MDPI containment implementation .	Strong recommendation: All countries should complete poliovirus-material inventories, destroy or transfer nonessential materials, and certify poliovirus-essential facilities under recognized containment standards.
Maintain eradication strategy through 2026.	Reinforced by continued WPV1 detection in Afghanistan and Pakistan and ongoing cVDPV2 outbreaks GPEI surveillance plan PDF .	Strong recommendation: Continue full implementation of the Global Polio Eradication Strategy 2022–2026, prioritizing WPV1 interruption and rapid cVDPV2 outbreak control.
Maintain AFP surveillance.	Updated by the 2026 AFP surveillance guidance replacing the 2022 version Global AFP guidance 2026 .	Strong recommendation: Use the 2026 AFP guidance as the current operational standard for AFP detection, investigation, laboratory testing, and reporting.

Original Recommendation	New Evidence Impact	Updated Recommendation
Use environmental surveillance to supplement AFP surveillance.	UK evidence shows environmental surveillance detected cVDPV2 and later a single orphan VDPV2 detection without evidence of local transmission GOV.UK environmental surveillance .	Strong recommendation: Expand or maintain environmental surveillance in high-risk, outbreak-affected, previously infected, or importation-risk areas.
Use outbreak-response vaccination for detected poliovirus transmission.	Reinforced by WHO prequalification of an additional novel OPV in February 2026 and evidence that wider nOPV2 use has contributed to declining cVDPV2 emergences, though transmission persists WHO polio health-topic page , GPEI surveillance plan PDF .	Strong recommendation: Deploy WHO-recommended outbreak-response vaccines rapidly and achieve high coverage, using nOPV2 or other appropriate vaccines according to poliovirus type and WHO/GPEI guidance.
Improve surveillance equity.	The March 2026 IHR Emergency Committee emphasized equitable coverage of all populations for timely detection WHO IHR Emergency Committee statement .	Strong recommendation: Surveillance plans must explicitly cover mobile, underserved, conflict-affected, border, migrant, and hard-to-reach populations.
Strengthen molecular sequencing and vaccine-quality systems.	WHO 2026 biological-standardization documents support sequencing reference reagents and note increasing importance of containment as the world transitions from OPV to IPV use WHO sequencing reference reagents PDF , WHO SAGE 2026 PDF .	Conditional recommendation: Countries and reference laboratories should expand sequencing capacity and adopt WHO reference reagents where feasible, prioritizing high-risk and outbreak settings.

Original Recommendation	New Evidence Impact	Updated Recommendation
Engage stakeholders in polio guideline development and implementation.	WHO's 2025 youth-engagement case study is not polio-specific but provides a transferable model for inclusive guideline development WHO publication .	Conditional recommendation: Future polio guidance should incorporate meaningful community, caregiver, adolescent, and youth engagement, particularly in vaccine confidence, outbreak communication, and equity planning.

Practical WHO-Based Polio Management Algorithm

A. Suspected case recognition and diagnosis

1. Suspect poliomyelitis in acute flaccid paralysis

- Treat AFP as a public-health emergency.
- Notify local and national surveillance authorities immediately.

2. Investigate rapidly

- Use national AFP surveillance procedures aligned with the 2026 global AFP guidance.
- Collect appropriate clinical, epidemiological, vaccination, travel, and contact information.
- Ensure virological testing through the polio laboratory network.

3. Classify and respond

- Distinguish WPV, VDPV, Sabin-like virus, or non-polio AFP through laboratory confirmation and sequencing where available.
- Initiate outbreak-risk assessment if poliovirus is detected.

B. Clinical treatment and supportive care

1. Provide supportive clinical care

- Assess respiratory function.
- Manage complications.
- Provide rehabilitation and mobility support.
- Prevent secondary complications.

2. No curative antiviral therapy identified

- WHO polio guidance in the supplied sources focuses on prevention, surveillance, outbreak response, and containment.

C. Public-health management

1. Vaccination

- Maintain high routine immunization coverage.
- Conduct outbreak-response vaccination rapidly when indicated.
- Use WHO-prequalified vaccine tools, including novel OPV where appropriate.

2. Surveillance

- Maintain sensitive AFP surveillance.
- Use environmental surveillance in high-risk or outbreak settings.
- Document surveillance gaps, corrective actions, and performance improvements.

3. Containment

- Inventory all poliovirus materials.
- Destroy, transfer, or contain materials according to WHO/GPEI containment standards.
- Certify poliovirus-essential facilities through recognized containment pathways.

4. Communication and engagement

- Use trusted local messengers.
 - Address vaccine confidence.
 - Include communities, caregivers, adolescents, and youth in future guideline and implementation planning.
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Final WHO-Based Guideline Position as of 29 April 2026

1. WHO/GPEI polio guidance as of 29 April 2026 is centered on **eradication strategy, vaccination, AFP and environmental surveillance, outbreak response, laboratory sequencing, and poliovirus containment**.
2. The five polio guidance documents synthesized here are:
 - **Guidelines for implementation of laboratory containment of wild poliovirus;**
 - **Global Polio Eradication Strategy 2022–2026;**
 - **Global Polio Surveillance Action Plan 2025–2026;**
 - **Global Guidelines for AFP Surveillance, 2026 update;**
 - **Global Poliovirus Containment Action Plan 2022–2024.**
3. New evidence through 29 April 2026 reinforces that **WPV1 persists in Afghanistan and Pakistan, cVDPV2 remains a multicountry threat**, and **environmental surveillance and sequencing are increasingly important** [GPEI surveillance plan PDF](#), [GOV.UK environmental surveillance](#).
4. WHO's **13 February 2026** prequalification of an additional novel oral polio vaccine strengthens outbreak-response capacity [WHO polio health-topic page](#).
5. The requested WHO meaningful-youth-engagement case study exists, but it concerns the **WHO Abortion care guideline (2022)**, not polio; its methods should be considered for future polio guideline development and community engagement [WHO publication](#).